INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Erlotinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00219
Version 6

Continuous daily tablets

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Erlotinib blocks abnormal EGFR growth signals in selected lung cancers. Treatment aims to control advanced cancer and delay progression.

EGFR TYROSINE KINASE INHIBITOR

Erlotinib

Blocks abnormal EGFR growth signals

Activating EGFR mutation identified → EGFR signalling blocked → Cancer growth slowed

Erlotinib blocks the EGFR tyrosine kinase signal that can drive growth in cancers with activating EGFR mutations. Effects on normal skin and bowel cells also contribute to side effects.

Report diarrhoea, rash, breathing changes and eye symptoms promptly.

Your treatment aim

Which setting?

NCCP 00219a is first-line EGFR-mutated advanced non-small-cell lung cancer (NSCLC); 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

NCCP 00219 v6 · official regimen · EMA Tarceva product information

Duration

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

What studies tell us

In a study of previously untreated EGFR-mutated advanced lung cancer, erlotinib delayed progression compared with chemotherapy. The final survival comparison did not show a statistically significant difference. The study population and subsequent treatments matter; ask how the evidence applies to your exact setting.

Your own decision

Ask how the study population matches your situation and discuss alternatives, harms and remaining uncertainty.

OPTIMAL primary PFS, Zhou et al. 2011 · OPTIMAL final OS, Zhou et al. 2015 · SATURN maintenance trial, Cappuzzo et al. 2010 · BR.21 previously treated NSCLC, Shepherd et al. 2005

Evidence for this treatment setting

The mutation and prior treatment matter.

Study groups cannot predict your individual outcome or compare treatments from different trials.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00219 v6 · official regimen · EMA Tarceva product information

THE WHOLE TREATMENT JOURNEY

Erlotinib monotherapy: treatment and checks

Your treatment pattern

Continuous daily tablets

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Activating-EGFR advanced NSCLC

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment NCCP 00219 · SMPC	Confirm indication and prescribed dose	Daily erlotinib; no planned rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree next treatment or follow-up
Blood tests NCCP 00219 · SMPC	Blood count, kidney and liver profile	Every 60 days; earlier if clinically needed	Continue every 60 days	Extra tests for dehydration or liver problems	Follow unresolved abnormalities
Symptoms and medicines NCCP 00219 · SMPC	Review lungs, bowel, skin, eyes and medicines	Report bowel, skin or eye symptoms early	Check adherence, supply and interactions	Confirm recovery before restart	Continue advice for unresolved symptoms
Cancer assessment NCCP 00219 · SMPC	Confirm EGFR, stage and prior treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

What each check is for

1



Confirm eligibility and the prescription

Before the first dose

The mutation and treatment setting determine applicability.

Ask / **act:** Document exact indication, dose and initial investigations.

2



Early bowel and skin review

From the first dose and early reviews

Early support can prevent dehydration and uncontrolled skin toxicity.

Ask / **act:** Record grade, duration, supportive treatment and recovery; apply medicine-specific thresholds.

3

New respiratory symptoms

At any time

Possible interstitial lung disease (ILD) requires urgent investigation.

Ask / **act:** Arrange urgent assessment and an authorised medicine plan.

4



Benefit and next-plan review

At planned cancer assessments or significant toxicity

Continuous treatment is not an unlimited automatic prescription.

Ask / **act:** Agree continuation, hold, restart, change or follow-up.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Tests and symptoms guide safe treatment decisions.

Before starting

Confirm the activating EGFR result, exact indication, previous treatment and prescription. Check blood count, kidneys and liver. Review lungs, bowel, skin, eyes, all medicines and pregnancy precautions.

During treatment

NCCP specifies blood count, kidney and liver tests every 60 days. Diarrhoea or dehydration needs extra kidney and electrolyte tests, including potassium. Liver problems or interacting medicines can require closer tests. Scans follow the Consultant’s plan.

Taking erlotinib

Follow your individual prescription.

Daily tablets

Take at least 1 hour before or 2 hours after food. If a dose is missed, do not replace it; take the usual next dose. If you vomit within a few hours after taking it, do not repeat the dose. Ask pharmacy before changing smoking habits or adding medicines for stomach acid.

Dose changes

Dose reductions use 150 → 100 → 50 mg daily. Recovery and restart depend on the specific side effect; only the team may authorise a restart. Do not increase the dose to compensate for smoking or acid-suppressing medicines.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily tablets, checks and decisions

No dates or personal information are needed. Confirm your own appointments with the team.

Treatment or check	Timing in this view
Preparation	**Before the first dose** Confirm the activating EGFR test, indication, prescription and baseline tests.
Daily oral treatment	**Early treatment review → Ongoing daily treatment → Benefit / toxicity review** The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.
Tests and symptoms	**Early treatment review → Ongoing daily treatment → Benefit / toxicity review → Treatment change or completion** NCCP specifies blood count, kidney and liver tests every 60 days. Diarrhoea or dehydration needs extra kidney and electrolyte tests, including potassium. Liver problems or interacting medicines can require closer tests. Scans follow the Consultant's plan.
Next treatment plan	**Treatment change or completion** Review response, side effects and whether to continue, change or stop treatment.

Take at least 1 hour before or 2 hours after food. If a dose is missed, do not replace it; take the usual next dose. If you vomit within a few hours after taking it, do not repeat the dose. Ask pharmacy before changing smoking habits or adding medicines for stomach acid.

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Living with treatment

Report symptoms early and keep contact details available.

Diarrhoea and fluids

Keep the prescribed anti-diarrhoea plan available from the first dose. Report loose stools early and keep drinking if able. Persistent or severe diarrhoea, weakness, inability to drink or much less urine requires urgent advice. The team decides whether to hold treatment and when it is safe to restart.

Skin, nails and mouth

Use the agreed skin, nail and mouth-care plan. Report rash, painful nails or mouth sores early. Emollients and sun protection may help; the team decides whether creams, antibiotics or other treatment are needed. Blistering or peeling skin needs urgent assessment.



Nausea medicines

Erlotinib has minimal-to-low oral emetic risk. Agree any as-needed anti-sickness medicine with the team. Routine infusion premedication is not required.

NCCP 00219 v6 · official regimen · NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 (2025), oral tables · NCCP NCIS antiemetic medicines V8 (2025); parenteral scope



Medicine interactions

Give pharmacy all medicines and supplements. Strong CYP3A4 inducers or inhibitors, grapefruit and medicines for stomach acid can change exposure. Avoid proton pump inhibitor combinations unless the specialist resolves the plan. Necessary antacids require source-specific separation; ask for written timing. Warfarin needs extra blood-clotting checks. Smoking reduces exposure; get cessation support and do not change your cancer dose yourself.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Use effective contraception during treatment and for at least 2 weeks after the final dose. Do not breastfeed during treatment or for at least 2 weeks after the final dose. Discuss fertility and future pregnancy plans before starting.

When to get urgent help

Do not wait for the next appointment.

Call urgently

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

When you call

Give the medicine name, last dose and symptoms. Tell the team if you can drink, pass urine and manage usual activities. Do not delay calling while gathering information.

When treatment changes

The team authorises continuation, interruption, restart or stopping.

Before restarting

Confirm symptom recovery, blood results, the prescribed dose and the next review.

When treatment ends

Agree follow-up, unresolved symptom care and reproductive precautions.

Practical support

Diarrhoea and fluids

Skin, nails and mouth

Nausea medicines

Medicine interactions

Pregnancy, fertility and breastfeeding

Taking tablets / attending treatment

Practical treatment details

Supplies and visits

Arrange the next prescription before tablets run out.

The oral regimen itself does not require an infusion line or pump.

Questions to ask

Explain the plan back: Explain when you take the daily tablet and how food affects it.,What will you do after a missed dose?,Which symptoms require an urgent call?,Who authorises a treatment hold or restart?

Which treatment this guide covers NCCP 00219a is first-line EGFR-mutated advanced non-small-cell lung cancer (NSCLC); 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Erlotinib alone · daily oral treatment

Active medicines: erlotinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Keep the prescribed anti-diarrhoea plan available from the first dose. Report loose stools early and keep drinking if able. Persistent or severe diarrhoea, weakness, inability to drink or much less urine requires urgent advice. The team decides whether to hold treatment and when it is safe to restart.
- Use the agreed skin, nail and mouth-care plan. Report rash, painful nails or mouth sores early. Emollients and sun protection may help; the team decides whether creams, antibiotics or other treatment are needed. Blistering or peeling skin needs urgent assessment.
- Erlotinib has minimal-to-low oral emetic risk. Agree any as-needed anti-sickness medicine with the team. Routine infusion premedication is not required.
- Give pharmacy all medicines and supplements. Strong CYP3A4 inducers or inhibitors, grapefruit and medicines for stomach acid can change exposure. Avoid proton pump inhibitor combinations unless the specialist resolves the plan. Necessary antacids require source-specific separation; ask for written timing. Warfarin needs extra blood-clotting checks. Smoking reduces exposure; get cessation support and do not change your cancer dose yourself.
- Avoid pregnancy. Use effective contraception during treatment and for at least 2 weeks after the final dose. Do not breastfeed during treatment or for at least 2 weeks after the final dose. Discuss fertility and future pregnancy plans before starting.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Which treatment this guide covers National Cancer Control Programme (NCCP) 00219a is first-line EGFR-mutated advanced non-small-cell lung cancer (NSCLC); 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

NCCP 00219 · VERSION 6

Erlotinib monotherapy

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Daily EGFR-targeted tablets · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Lungs

erlotinib

New cough or breathlessness needs urgent assessment.

2 Bowel

erlotinib

Diarrhoea can cause dehydration; follow the symptom plan.

3 Skin and nails

erlotinib

Report rash, painful nails or peeling skin.

4 Eyes

erlotinib

Eye pain or blurred vision needs prompt review.

5 Liver

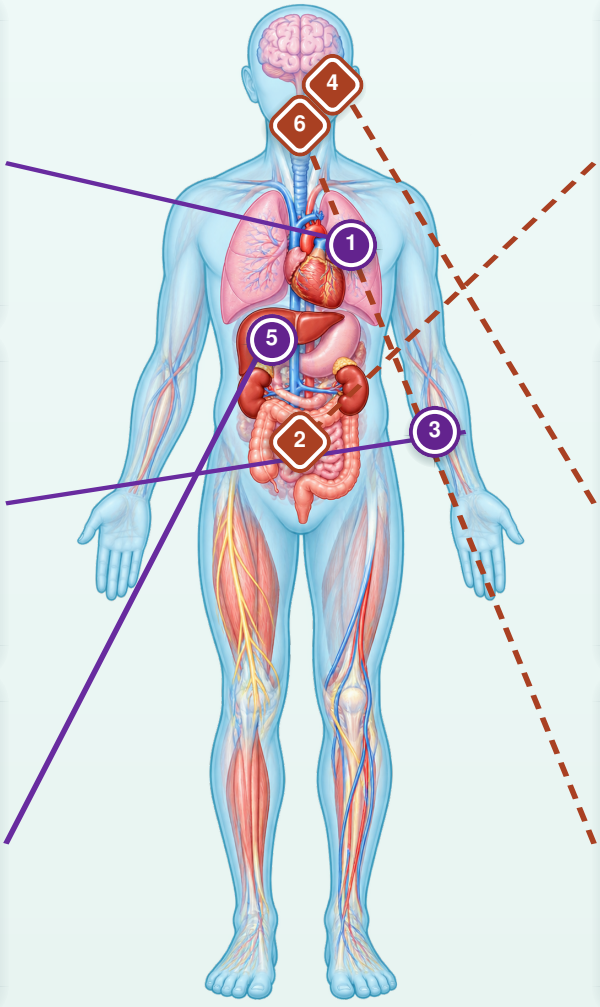
erlotinib

Blood tests check for liver injury.

6 Mouth

erlotinib

Report soreness or difficulty eating.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Diarrhoea	☐	☐	☐	
Skin / nail changes	☐	☐	☐	
Mouth soreness / ulcers	☐	☐	☐	
New cough or breathlessness	☐	☐	☐	
Eye pain or blurred vision	☐	☐	☐	
Fatigue	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions. Emergency symptoms: 112 / 999.

Carry your treatment details

Which treatment this guide covers NCCP 00219a is first-line EGFR-mutated advanced NSCLC; 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Erlotinib monotherapy

NCCP 00219 v6

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

https://sactcheck.com/patient/00219/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.26 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00219 v6 · official regimen**
   https://healthservice.hse.ie/documents/6938/219_Erlotinib_monotherapy.pdf
2. **EMA Tarceva product information**
   https://www.ema.europa.eu/en/documents/product-information/tarceva-epar-product-information_en.pdf
3. **NCCP V6 oral minimal-low: individual as-required antiemetics; NCIS V8 is parenteral and does not prescribe an oral course**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS antiemetic medicines V8 (2025); parenteral scope**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **OPTIMAL primary PFS, Zhou et al. 2011**
   https://pubmed.ncbi.nlm.nih.gov/21783417/
6. **OPTIMAL final OS, Zhou et al. 2015**
   https://pubmed.ncbi.nlm.nih.gov/26141208/
7. **SATURN maintenance trial, Cappuzzo et al. 2010**
   https://pubmed.ncbi.nlm.nih.gov/20493771/
8. **BR.21 previously treated non-small-cell lung cancer (NSCLC), Shepherd et al. 2005**
   https://pubmed.ncbi.nlm.nih.gov/16014882/